

CIOMS FORM I
SUSPECT ADVERSE REACTION REPORT

Council for International Organizations of Medical Sciences (CIOMS) — Standard Reporting Form

I. REACTION INFORMATION

1. PATIENT INITIALS M.K.	1a. COUNTRY USA	2. DATE OF BIRTH 14-MAY-1967	2a. AGE 58 YRS	3. SEX FEMALE	3a. WEIGHT 68 kg (150 lbs)
4. REACTION ONSET DATE 08-NOV-2025		5. TODAY'S DATE 12-NOV-2025		8-12. CHECK ALL APPROPRIATE TO ADVERSE REACTION: [] PATIENT DIED [] LIFE-THREATENING [X] INVOLVED OR PROLONGED INPATIENT HOSPITALIZATION (Admitted: 10-NOV-2025) [] PERSISTENT/SIGNIFICANT DISABILITY [] CONGENITAL ANOMALY [X] OTHER MEDICALLY IMPORTANT CONDITION: YES (Acute Drug-Induced Liver Injury)	
6. DESCRIBE REACTION(S) (including relevant medical history, signs, symptoms, and clinical course): 58-year-old female patient (M.K.) with medical history of essential hypertension (5 yrs) and type 2 diabetes mellitus (4 yrs) initiated Cardioril 20 mg once daily on 15-OCT-2025. On 08-NOV-2025 (~24 days post-initiation), patient presented with progressive fatigue, anorexia, severe generalized pruritus, dark brown urine, and visible scleral icterus with painless clinical jaundice. Outpatient chemistry panel showed profound transaminitis (ALT 540 U/L, AST 420 U/L) and total bilirubin 4.8 mg/dL meeting Hy's Law criteria. Patient emergently admitted to MetroHealth Gastroenterology Service on 10-NOV-2025 for acute drug-induced liver injury (DILI). Viral hepatitis panel (anti-HAV IgM, HBsAg, anti-HCV, anti-HEV IgM) was non-reactive. Abdominal ultrasound revealed normal liver parenchyma without biliary ductal dilatation or cholelithiasis. Cardioril discontinued permanently on admission (10-NOV-2025). Outcome: Recovering with trending decrease in transaminases on serial daily testing.					

II. SUSPECT DRUG(S) INFORMATION

14. SUSPECT DRUG NAME (Brand & Generic) Cardioril (cardioril hydrochloride)	15. DAILY DOSE(S) 20 mg once daily (QD)	16. ROUTE OF ADMIN. Oral (tablet)	17. INDICATION(S) FOR USE Refractory Essential Hypertension
18. THERAPY DATES (start / stop) 15-OCT-2025 to 10-NOV-2025	19. THERAPY DURATION 26 days	20. LOT / BATCH NUMBER Lot #CR-2025-0981 (Exp: 08/2027)	21. ACTION TAKEN [X] Drug permanently withdrawn

III. CONCOMITANT DRUG(S) AND HISTORY

22. CONCOMITANT DRUG(S) AND DATES OF ADMINISTRATION (exclude drugs used to treat reaction): 1. Metformin HCl 500 mg PO BID (Indication: Type 2 Diabetes Mellitus; Start: 12-JAN-2021, Ongoing). 2. Amlodipine besylate 5 mg PO QD (Indication: Hypertension; Start: 05-MAR-2023, Ongoing). No over-the-counter NSAIDs, herbal supplements, or acetaminophen use reported. RELEVANT MEDICAL HISTORY: Essential hypertension (5 yrs), T2DM (4 yrs). No prior history of hepatobiliary disease, jaundice, or ethanol abuse.					
23. RELEVANT TESTS / LABORATORY DATA:	RESULT	UNITS	REF. RANGE	INTERPRETATION	DATE
Alanine Aminotransferase (ALT / SGPT)	540	U/L	7 – 56	CRITICAL ELEVATION (>9x ULN)	10-NOV-2025
Aspartate Aminotransferase (AST / SGOT)	420	U/L	10 – 40	CRITICAL ELEVATION (>10x ULN)	10-NOV-2025
Total Bilirubin	4.8	mg/dL	0.1 – 1.2	SEVERE HYPERBILIRUBINEMIA (Hy's Law)	10-NOV-2025
Alkaline Phosphatase (ALP)	210	U/L	44 – 147	ELEVATED	10-NOV-2025
Serum Creatinine	0.9	mg/dL	0.6 – 1.2	NORMAL	10-NOV-2025
Hepatitis A, B, C Serology (IgM, HBsAg, HCV-Ab)	Negative	-	Negative	NON-REACTIVE (Viral etiology ruled out)	11-NOV-2025

IV. MANUFACTURER / REPORTER INFORMATION

24a. NAME AND ADDRESS OF REPORTER Sarah Jenkins, MD, FACP Division of Gastroenterology & Hepatology MetroHealth Medical Center 2500 MetroHealth Dr, Chicago, IL 60609, USA	24b. MFR CONTROL NO. CR-2025-US-00891 24c. DATE RECEIVED BY MFR 12-NOV-2025	24d. HEALTH PROFESSIONAL? [X] YES [] NO 24e. OCCUPATION / SPECIALTY Physician / Gastroenterologist Tel: (312) 555-0188
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